

# Colorectal Cancer Screening Tests

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## Tracking Information

### Publication Number

100-3

### Manual Section Number

210.3

### Manual Section Title

Colorectal Cancer Screening Tests

### Version Number

7

### Effective Date of this Version

01/01/2023

### Implementation Date

02/27/2023

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## Description Information

### Benefit Category

Colorectal Cancer Screening Tests

**Please Note:** This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

### Item/Service Description

#### A. General

Sections 1861(s)(2)(R) and 1861(pp) of the Social Security Act (the Act) and regulations at 42 CFR 410.37 authorize Medicare coverage for screening colorectal cancer tests under Medicare Part B. The statute and regulations authorize the Secretary to add other tests and procedures (and modifications to tests and procedures for colorectal cancer screening) as the Secretary finds appropriate based on consultation with appropriate organizations.

### Indications and Limitations of Coverage

#### B. Nationally Covered Indications

## **1. Fecal Occult Blood Tests (FOBT) (effective January 1, 2004)**

Fecal occult blood tests (FOBTs) are generally divided into two types: immunoassay and guaiac types. Immunoassay (or immunochemical) fecal occult blood tests (iFOBT) use “antibodies directed against human globin epitopes. While most iFOBTs use spatulas to collect stool samples, some use a brush to collect toilet water surrounding the stool. Most iFOBTs require laboratory processing.

Guaiac fecal occult blood tests (gFOBT) use a peroxidase reaction to indicate presence of the heme portion of hemoglobin. Guaiac turns blue after oxidation by oxidants or peroxidases in the presence of an oxygen donor such as hydrogen peroxide. Most FOBTs use sticks to collect stool samples and may be developed in a physician’s office or a laboratory. In 1998, Medicare began reimbursement for guaiac FOBTs, but not immunoassay type tests for colorectal cancer screening. Since the fundamental process is similar for other iFOBTs, the Centers for Medicare & Medicaid Services evaluated colorectal cancer screening using immunoassay FOBTs in general.

Effective for dates of service on and after January 1, 2004, Medicare covers one screening FOBT per annum for the early detection of colorectal cancer. This means that Medicare will cover one gFOBT or one iFOBT at a frequency of every 12 months; i.e., at least 11 months have passed following the month in which the last covered screening FOBT was performed, for beneficiaries aged 50 years and older. The beneficiary completes the existing gFOBT by taking samples from two different sites of three consecutive stools; the beneficiary completes the iFOBT by taking the appropriate number of stool samples according to the specific manufacturer’s instructions. This screening requires a written order from the beneficiary’s attending physician. (“Attending physician” means a doctor of medicine or osteopathy (as defined in §1861(r)(1) of the Act) who is fully knowledgeable about the beneficiary’s medical condition, and who would be responsible for using the results of any examination performed in the overall management of the beneficiary’s specific medical problem.)

Effective January 1, 2023, the minimum age for FOBT is reduced to 45 years and older.

## **2. The Cologuard™ – Multi-target Stool DNA (sDNA) Test (effective October 9, 2014)**

Screening stool or fecal DNA (deoxyribonucleic acid, sDNA) testing detects molecular markers of altered DNA that are contained in the cells shed by colorectal cancer and pre-malignant colorectal epithelial neoplasia into the lumen of the large bowel. Through the use of selective enrichment and amplification techniques, sDNA tests are designed to detect very small amounts of DNA markers to identify colorectal cancer or pre-malignant colorectal neoplasia. The Cologuard™ – multi-target sDNA test is a proprietary in vitro diagnostic device that incorporates both sDNA and fecal immunochemical test techniques and is designed to analyze patients’ stool samples for markers associated with the presence of colorectal cancer and pre-malignant colorectal neoplasia.

Effective for dates of service on or after October 9, 2014, The Cologuard™ test is covered once every three years for Medicare beneficiaries that meet all of the following criteria:

- Age 50 to 85 years, and,

- Asymptomatic (no signs or symptoms of colorectal disease including but not limited to lower gastrointestinal pain, blood in stool, positive guaiac fecal occult blood test (gFOBT) or fecal immunochemical test (iFOBT)), and,
- At average risk of developing colorectal cancer (no personal history of adenomatous polyps, colorectal cancer, or inflammatory bowel disease, including Crohn's Disease and ulcerative colitis; no family history of colorectal cancers or adenomatous polyps, familial adenomatous polyposis, or hereditary nonpolyposis colorectal cancer).

Effective January 1, 2023, the minimum age for sDNA is reduced to 45 years and older.

### **3. Blood-based Biomarker Tests (effective January 19, 2021)**

Blood-based DNA testing detects molecular markers of altered DNA that are contained in the cells shed into the blood by colorectal cancer and pre-malignant colorectal epithelial neoplasia.

Effective for dates of service on or after January 19, 2021, a blood-based biomarker test is covered as an appropriate colorectal cancer screening test once every 3 years for Medicare beneficiaries when performed in a Clinical Laboratory Improvement Act (CLIA)-certified laboratory, when ordered by a treating physician and when all of the following requirements are met:

The patient is:

- age 50-85 years, and,
- asymptomatic (no signs or symptoms of colorectal disease including but not limited to lower gastrointestinal pain, blood in stool, positive guaiac fecal occult blood test or fecal immunochemical test), and,
- at average risk of developing colorectal cancer (no personal history of adenomatous polyps, colorectal cancer, or inflammatory bowel disease, including Crohn's Disease and ulcerative colitis; no family history of colorectal cancers or adenomatous polyps, familial adenomatous polyposis, or hereditary nonpolyposis colorectal cancer).

The blood-based biomarker screening test must have all of the following:

- Food and Drug Administration (FDA) market authorization with an indication for colorectal cancer screening; and,
- proven test performance characteristics for a blood-based screening test with both sensitivity greater than or equal to 74% and specificity greater than or equal to 90% in the detection of colorectal cancer compared to the recognized standard (accepted as colonoscopy at this time), as minimal threshold levels, based on the pivotal studies included in the FDA labeling.

Effective January 1, 2023, the minimum age for blood-based biomarker test is reduced to 45 years and older.

### **C. Nationally Non-Covered Indications**

All other indications for colorectal cancer screening not otherwise specified in the Act and regulations, or otherwise specified above remain nationally non-covered. Non-coverage

specifically includes:

(1) All screening sDNA tests, effective April 28, 2008, through October 8, 2014. Effective for dates of service on or after October 9, 2014, all other screening sDNA tests not otherwise specified above remain nationally non-covered.

(2) Screening computed tomographic colonography (CTC), effective May 12, 2009.

#### D. Other

N/A

(This NCD was last reviewed January 2023 and includes policy updates finalized in the Calendar Year 2023 Physician Fee Schedule final rule 87 FR69404)

#### Cross Reference

Also see NCD for Fecal Occult Blood Test (§[190.34](#) [↗](#)).

#### Claims Processing Instructions

[TN 11824 \(Medicare Claims Processing\)](#) [↗](#)

[TN 11865 \(Medicare Claims Processing\)](#) [↗](#)

[TN 12299 \(Medicare Claims Processing\)](#) [↗](#)

[TN 13248 \(Medicare Claims Processing\)](#) [↗](#)

[TN 13295 \(Medicare Claims Processing\)](#) [↗](#)

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## Transmittal Information

#### Transmittal Number

11824

#### Coverage Transmittal Link

<https://www.cms.gov/files/document/R11824NCD.pdf> [↗](#)

#### Revision History

09/2025 - Transmittal 13400 issued September 05, 2025, is being rescinded and replaced by Transmittal 13438, dated September 30, 2025, to add a new attachment to ensure the functionality of the NCD zip URL is working properly and to update the Policy section. All other information remains the same.

The purpose of this Change Request (CR) is to provide a maintenance update of ICD-10 coding changes specific to NCDs. ([TN 13438](#) [↗](#)) (CR14197)

08/2025 - The purpose of this Change Request (CR) is to provide a maintenance update of ICD-10 coding changes specific to NCDs. ([TN 13375](#) [↗](#)) (CR14197)

03/2025 - The purpose of this Change Request (CR) is to provide a quarterly maintenance update of ICD-10 coding conversions and other coding updates specific to National Coverage Determinations (NCDs). No policy is being changed as a result of these updates. ([TN 13097](#) [↗](#)) (CR13939)

10/2024 - The purpose of the Change Request (CR) is to provide a quarterly maintenance update of ICD-10 coding conversions and other coding updates specific to National Coverage Determinations(NCDs). No policy is being changed as a result of these updates. ([TN 12904](#)) (CR13828)

01/2024 - Transmittal 12350 issued November 03, 2023, is being rescinded and replaced by Transmittal 12440 dated January 3, 2024, to make changes to NCD 90.2, Next Generation Sequencing, spreadsheet to align with revisions being made to CR 13278. All other information remains the same. ([TN 12440](#)) (CR13391)

11/2023 - The purpose of this Change Request (CR) is to provide a quarterly maintenance update of ICD-10 coding conversions and other coding updates specific to National Coverage Determinations (NCDs). No policy is being changed as a result of these updates. ([TN 12350](#)) (CR13391)

10/2023 - The purpose of this Change Request (CR) is to provide a quarterly maintenance update of ICD-10 coding conversions and other coding updates specific to National Coverage Determinations (NCDs). No policy is being changed as a result of these updates. ([TN 12319](#)) (CR13391)

02/2023 - Transmittal 11824 issued January 27, 2023, is being rescinded and replaced by Transmittal 11865, dated, February 16, 2023, to add the Spanish version of MSN Message 18.29 to the IOM for publication 100-04. This correction does not make any revisions to the companion Pubs.100-02 or 100-03; all revisions are associated with Pub. 100-04. All other information remains the same. ([TN 11865](#)) (CR13017)

01/2023 - The purpose of this omnibus Change Request (CR) is to make contractors aware of policy updates resulting from changes specified in the Calendar Year (CY) 2023 Physician Fee Schedule (PFS) Final Rule (87 FR 69404), published in the Federal Register on 11/18/2022. The policy updates include removal of one selected National Coverage Determination (NCD) : Ambulatory Electroencephalographic (EEG) Monitoring (NCD 160.22). Separately, the policy updates also include policies to expand colorectal cancer screening coverage by 1) reducing the minimum age for certain CRC screening tests from 50 to 45 years and 2) expanding the regulatory definition of CRC screening tests to include a follow-on screening colonoscopy after a Medicare covered non-invasive stool-based test returns a positive result. ([TN 11824](#)) (CR13017)

05/2021 - The purpose of this Change Request (CR) is to inform contractors that CMS has determined effective on January 19, 2021 blood-based biomarker test is an appropriate colorectal cancer screening test based on specific criteria.The Federal government creates NCDs that are binding on the MACs who review and/or adjudicate claims, make coverage determinations, and/or payment decisions, and also binds quality improvement organizations, qualified independent contractors, the Medicare appeals council, and Administrative Law Judges (ALJs) (see 42 Code of Federal Regulations (CFR) section 405.1060(a)(4) (2005)). An NCD that expands coverage is also binding on a Medicare advantage organization. In addition, an ALJ may not review an NCD. (See section 1869(f)(1)(A)(i) of the Social Security Act.) ([TN 10818](#)) (CR12280)

02/2020 - This Change Request (CR) constitutes a maintenance update of ICD-10 conversions and other coding updates specific to NCDs. These NCD coding changes are the result of newly available codes, coding revisions to NCDs released separately, or coding feedback received.

Previous NCD coding changes appear in ICD-10 quarterly updates that can be found at: <https://www.cms.gov/Medicare/Coverage/CoverageGenInfo/ICD10.html>, along with other CRs implementing new policy NCDs. Edits to ICD-10 and other coding updates specific to NCDs will be included in subsequent quarterly releases and individual CRs as appropriate. No policy-related changes are included with the ICD-10 quarterly updates. Any policy-related changes to NCDs continue to be implemented via the current, long-standing NCD process. (TN 2427) (CR11491)

02/2018 - Transmittal 2033, dated February 16, 2018, is being rescinded and replaced by Transmittal 2039, dated, February 28, 2018 to correct instructions in business requirement 7, NCD210.3, Colorectal Cancer Screening, and its accompanying spreadsheet. All other information remains the same. (TN 2039) (CR10473)

02/2018 - This Change Request (CR) constitutes a maintenance update of International Code of Diseases, Tenth Revision (ICD-10) conversions and other coding updates specific to National Coverage Determinations (NCDs). These NCD coding changes are the result of newly available codes, coding revisions to NCDs released separately, or coding feedback received.

Previous NCD coding changes appear in ICD-10 quarterly updates that can be found at: <https://www.cms.gov/Medicare/Coverage/CoverageGenInfo/ICD10.html>, along with other CRs implementing new policy NCDs. Edits to ICD-10 and other coding updates specific to NCDs will be included in subsequent quarterly releases and individual CRs as appropriate. No policy-related changes are included with the ICD-10 quarterly updates. Any policy-related changes to NCDs continue to be implemented via the current, long-standing NCD process. (TN 2033) (CR10473)

02/2017 - This change request (CR) is the 10th maintenance update of ICD-10 conversions and other coding updates specific to national coverage determinations (NCDs). These NCD coding changes are the result of newly available codes, coding revisions to NCDs released separately, or coding feedback received.

Previous NCD coding changes appear in ICD-10 quarterly updates as follows: CR7818, CR8109, CR8197, CR8691, CR9087, CR9252, CR9540, CR9631, and CR9751, as well as in CRs implementing new policy NCDs. Edits to ICD-10 and other coding updates specific to NCDs will be included in subsequent, quarterly releases and individual CRs as appropriate. No policy-related changes are included with the ICD-10 quarterly updates. Any policy-related changes to NCDs continue to be implemented via the current, long-standing NCD process. (TN 1792) (CR9861)

06/2016 - Transmittal 1665, dated May 13, 2016, is being rescinded and replaced by Transmittal 1672 to: (1) 9631.1: Remove FISS responsibility and 1st sentence; (2) 9631.2: Remove additional procedure codes, including 0075T, 0076T; (3) 9631.4: Revise descriptor of dx L59.8; (4) 9631.6: Add deletion of dx C49.10, C65.9, remove deletion of dx C54.9, remove deletion of invalid dx C47.90; (5) 9631.8: Remove deletion of invalid dx C51.29, replace with



deletion of dx C50.029; (6) 9631.9: Add deletion of 0V504ZZ, 0V500ZZ, override capability, and contractor discretion verbiage. All other information remains the same. ([TN 1672](#)) (CR9631)

05/2016 - This change request (CR) is the 7th maintenance update of ICD-10 conversions and other coding updates specific to national coverage determinations (NCDs). The majority of the NCDs included are a result of feedback received from previous ICD-10 NCD CR7818, CR8109, CR8197, CR8691, CR9087, CR9252, and CR9540. Some are the result of revisions required to other NCD-related CRs released separately.

Edits to ICD-10 and other coding updates specific to NCDs will be included in subsequent, quarterly releases as needed. No policy-related changes are included with these updates. Any policy-related changes to NCDs continue to be implemented via the current, long-standing NCD process. ([TN 1665](#)) (CR9631)

04/2016 - Transmittal 1630, dated February 26, 2016, is being rescinded and replaced by Transmittal 1658 to (1) remove duplicate spreadsheet NCD210.3, (2) add missing spreadsheet NCD20.33, (3) add B/MAC to requirement 3 at request of WPS/B, (4) rename the spreadsheet titles, and, (5) provide a link to the attached spreadsheets for more efficient ease of reference and accessibility. All other information remains the same. ([TN 1658](#)) (CR9540)

12/2015 - This change request (CR) is the 3rd maintenance update of ICD-10 conversions/updates specific to national coverage determinations (NCDs). The majority of the NCDs included are a result of feedback received from previous ICD-10 NCD CR7818, CR8109, CR8197, CR8691, & CR9087. Some are the result of revisions required to other NCD-related CRs released separately that included ICD-10 coding. Implementation date: 01/04/2016 Effective date: 10/1/2015. ([TN 1580](#)) (CR9252)

08/2015 - This change request (CR) is the 3rd maintenance update of ICD-10 conversions/updates specific to national coverage determinations (NCDs). The majority of the NCDs included are a result of feedback received from previous ICD-10 NCD CR7818, CR8109, CR8197, CR8691, & CR9087. Some are the result of revisions required to other NCD-related CRs released separately that included ICD-10 coding. These updates do not expand, restrict, or alter existing coverage policy. Implementation date: 01/04/2016 Effective date: 10/1/2015. ([TN 1537](#)) (CR 9252)

08/2015 - The purpose of this Change Request (CR) is effective for claims with dates of service on or after October 9, 2014, contractors shall recognize new HCPCS code G0464 (colorectal cancer screening; stool-based DNA and fecal occult hemoglobin) as a covered service. Effective date: 10/09/2014 Implementation date: 09/08/2015. ([TN 183](#)) (CR9115)

10/2014 - Effective for dates of services on and after January 27, 2014, contractors shall pay claims for Ultrasound screening for AAA and screening FOBTs, per the modified requirements in 42 CFR 410.9 and 410.37. Effective date: 01/27/2014. Implementation date: 11/18/2014. ([TN 176](#)) (CR8881)

05/2014 - CMS translated the information for this policy from ICD-9-CM/PCS to ICD-10-CM/PCS according to HIPAA standard medical data code set requirements and updated any necessary and related coding infrastructure. These updates do not expand, restrict, or alter

existing coverage policy. Implementation date: 10/06/2014 Effective date: 10/1/2015. ([TN 1388](#)) (CR 8691)

01/2013 - CMS translated the information for this policy from ICD-9-CM/PCS to ICD-10-CM/PCS according to HIPAA standard medical data code set requirements and updated any necessary and related coding infrastructure. These updates do not expand, restrict, or alter existing coverage policy. Implementation date: 04/01/2013 Effective date: 10/1/2015. ([TN 1165](#)) (CR 8109)

08/2009 - CMS determines that the current evidence is inadequate to conclude that CTC is an appropriate colorectal cancer screening test under section 1861(pp)(1) of the Social Security Act. Therefore, effective May 12, 2009, CTC for colorectal cancer screening remains nationally non-covered. Effective date 05/12/2009. Implementation date 09/08/2009. ([TN 105](#)) (CR6578)

08/2008 - Following reconsideration of the current national coverage determination (NCD) for colorectal cancer screening, CMS proposes not to expand the colorectal cancer screening benefit to include coverage of PreGen-Plus, a commercially available screening DNA stool test. The FDA determines that this test requires premarket review and approval. A subsequent request for reconsideration will be considered once FDA approval is obtained. Effective date 04/28/2008. Implementation date 08/25/2008. ([TN 92](#)) (CR6145)

12/2003 - Expanded Medicare coverage for screening for early detection of colorectal cancer by adding additional fecal occult blood test (iFOBT, immunoassay-based) that can be used as alternative to existing gFOBT, guaiac-based test. Medicare coverage continues to allow one FOBT per year for beneficiaries aged 50 and over. Effective date 1/01/04. Implementation date 1/05/2004 for coverage & HCPCS codes and 4/05/2004 for frequency edits. ([TN 5](#)) (CR 2996)

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## National Coverage Analyses (NCAs)

This NCD has been or is currently being reviewed under the National Coverage Determination process. The following are existing associations with NCAs, from the National Coverage Analyses database.

- [Original Consideration for Screening DNA Stool Test for Colorectal Cancer \(CAG-00144N\)](#)
- [Original Consideration for Screening Immunoassay Fecal-Occult Blood Test \(CAG-00180N\)](#)
- [Original Consideration for Screening for Colorectal Cancer - Stool DNA Testing \(CAG-00440N\)](#)
- [First reconsideration for Screening for Colorectal Cancer-Non-Invasive Biomarker Tests \(CAG-00440R\)](#)



- [Original Consideration for Screening for Colorectal Cancer - Blood-Based Biomarker Tests \(CAG-00454N\)](#)

## Coding Analyses for Labs (CALs)

This NCD has been or is currently being reviewed under the National Coverage Determination process. The following are existing associations with CALs, from the Coding Analyses for Labs database.

- [Original Consideration for Prothrombin Time and Fecal Occult Blood \(Revision of ICD-9-CM Codes for Injury to Gastrointestinal Tract\) \(CAG-00187N\)](#)

## Additional Information

### Other Versions

| Title                             | Version | Effective Between       |  |
|-----------------------------------|---------|-------------------------|-------------------------------------------------------------------------------------|
| Colorectal Cancer Screening Tests | 7       | 01/01/2023 - N/A        | You are here                                                                        |
| Colorectal Cancer Screening Tests | 6       | 01/19/2021 - 01/01/2023 | <a href="#">View</a>                                                                |
| Colorectal Cancer Screening Tests | 5       | 10/09/2014 - 01/19/2021 | <a href="#">View</a>                                                                |
| Colorectal Cancer Screening Tests | 4       | 01/27/2014 - 10/09/2014 | <a href="#">View</a>                                                                |
| Colorectal Cancer Screening Tests | 3       | 05/12/2009 - 01/27/2014 | <a href="#">View</a>                                                                |
| Colorectal Cancer Screening Tests | 2       | 04/28/2008 - 05/12/2009 | <a href="#">View</a>                                                                |
| Colorectal Cancer Screening Tests | 1       | 01/01/2004 - 04/28/2008 | <a href="#">View</a>                                                                |

